



P37 · 12 CARDS, ONE PER WELL-CHILD VISIT THROUGH 2Y

Pediatrician Visit Prep Cards

Twelve cards, one per well-baby visit from newborn through age two. Each card lists what the pediatrician usually checks at that visit, what you can prepare in advance, what to ask, what to write down after the appointment, and red flags to watch for between visits. A "when to call between visits" page rounds out the deck.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A note from the maker

The well-baby visits arrive faster than you expect. By the time you have made the appointment, gotten the car seat in, and figured out which onesie they will not throw up on this morning, you have forgotten the four questions you wanted to ask.

These cards are the prep work. One card per visit. You fill in the "what to bring" section the night before. You fill in the "what they said" section right after, before you drive home.

After two years, you have a record. Of what was measured. Of what they recommended. Of what you asked and what you got back. It is yours.

With care, Maya

This is a planning tool, not medical advice. For specific concerns about your child's health, please follow your pediatric provider's guidance. The cards do not name vaccines or medications. Your pediatrician will tell you the current schedule.

How to use the cards

- Print on cardstock if you can. Plain paper is fine.
- Each card is one well-baby visit. The card for the next visit goes in the diaper bag.
- Fill in the “what to prepare” and “questions I want to ask” sections the night before.
- Fill in the “what they said” and “what to do next” sections right after the visit, before you drive home.
- Keep the filled-in cards in a folder. After two years, you have a record.

A small detail: pediatric appointments are often shorter than the parent’s questions. The card helps you ask the most important ones first.

What to expect at every visit, in general

This is the rhythm that mostly applies to every well-baby visit. Specifics vary by office.

The intake. A nurse or medical assistant measures: weight, length (later height), head circumference (in the first two years). They may ask about feeding, sleep, and any concerns.

The exam. The pediatrician listens to heart and lungs. Looks in eyes, ears, mouth, throat. Feels the abdomen. Checks the hips. Checks the fontanelles (the soft spots) in babies. Checks reflexes in babies. Checks development through observation and questions.

The development conversation. They ask what your kid is doing. Movement, language, social. They may go through a checklist with you. Bring honesty, not what you wish were true.

The recommendations. Feeding guidance. Sleep guidance. Safety guidance. Vaccines, if it is a visit when vaccines are scheduled.

The next visit. They tell you when to come back. Sometimes you schedule on the way out.

A note about vaccines. Many well-baby visits include vaccines. Your pediatrician follows a recommended schedule. The schedule changes over time as research updates it. This planner does not name specific vaccines, because the schedule is what your pediatrician knows. Ask. Write down what was given today. The cards have a line for it.

Card 1, the newborn / first-week visit

Approximate age: 3 to 5 days after discharge from the hospital.

Why this visit happens so early: Newborns lose weight in the first few days after birth and then start gaining it back. The pediatrician wants to see that the gain has started. Jaundice (yellowing) is also checked at this visit.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ Jaundice (skin and eye color, sometimes a blood test)
- ☐ The cord stump
- ☐ Heart and lungs
- ☐ Reflexes
- ☐ Feeding (breast / bottle / both): how often, how long, how it is going
- ☐ Wet and dirty diaper count over the last 24 hours
- ☐ The parent's mood and energy

WHAT TO PREPARE:

- ☐ How many wet diapers in the last 24 hours: _____
- ☐ How many dirty diapers in the last 24 hours: _____
- ☐ How often baby is feeding: _____
- ☐ Any color changes you noticed (skin, eyes, lips): _____
- ☐ Your top concern, in one sentence: _____

WHAT TO ASK:

- What is their weight relative to their birth weight?
- Is the jaundice normal for them, or do we need follow-up?
- Is the feeding pattern okay?
- When do I call you between now and the next visit?

WHAT THEY SAID:

Birth weight: _____
Today's weight: _____
Length: _____
Head circumference: _____

Notes from the exam:

What they recommended about feeding:

What they recommended about sleep position / safe sleep:

Next visit scheduled for: _____

RED FLAGS BEFORE THE NEXT VISIT (call the pediatrician):

- Any fever (any temperature elevation in a newborn warrants a call)
- Very few or no wet diapers
- Significant weight loss continuing
- Yellowing of the skin or eyes that is getting worse
- Trouble breathing, fast or noisy breathing
- Hard to wake, or unusually limp
- Inconsolable crying for an extended stretch
- A sense that something is wrong

Card 2, the 1 to 2 week visit

Approximate age: 10 to 14 days after birth.

Why this visit: Continued growth check. Feeding troubleshooting if needed. Parent check-in.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ Has the baby regained their birth weight?
- ☐ Jaundice resolved or resolving
- ☐ The cord stump, if still attached
- ☐ Heart and lungs
- ☐ Reflexes
- ☐ Feeding details
- ☐ The parent (this is the visit where postpartum mood often comes up)

WHAT TO PREPARE:

- ☐ Wet diapers, last 24 hours: _____ Dirty: _____
- ☐ Feeding pattern, last 24 hours: _____
- ☐ Anything new I noticed this week: _____
- ☐ My own mood: _____
- ☐ My top question: _____

WHAT TO ASK:

- Are they back to birth weight?
- Is the feeding pattern on track?
- (If breastfeeding) Should I see a lactation consultant for anything you noticed?
- Are there any reflexes you want me to keep an eye on?
- When is the next visit?

WHAT THEY SAID:

Weight today: _____
Length: _____
Head circumference: _____

Notes:

What they said about feeding:

What they said about the cord:

What they said when I mentioned my own mood:

Next visit: _____

RED FLAGS:

- Any fever
- Weight not coming back up
- Concerning rash
- Trouble breathing
- Hard to wake, unusually limp
- A change that worries you

Card 3, the 1 month visit

Approximate age: 4 weeks.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference, plotted on the growth chart
- ☐ Heart and lungs
- ☐ Reflexes (Moro, rooting, grasp, etc.)
- ☐ The fontanelles
- ☐ Hip stability
- ☐ Development: lifting head briefly, focusing on faces, beginning to smile
- ☐ Feeding
- ☐ Sleep patterns

WHAT TO PREPARE:

- ☐ Diaper count, last 24 hours: _____ wet _____ dirty
- ☐ Feeding pattern: _____
- ☐ Anything new (smiles, sounds, movements, dislikes): _____
- ☐ My top question: _____

WHAT TO ASK:

- How is their growth tracking?
- Anything I should be watching for between now and 2 months?
- Are there any vaccines at the 2-month visit I should know about ahead of time?
- (If breastfeeding) How long do you typically recommend exclusive breastfeeding?
- When is the next visit?

WHAT THEY SAID:

Weight: _____ Percentile: _____

Length: _____ Percentile: _____

Head: _____ Percentile: _____

Notes:

Development they noticed today:

Their feeding recommendation:

Their sleep recommendation:

Next visit: _____

RED FLAGS:

- Any fever
- Not feeding well
- Not gaining weight
- Eye that does not track or open well
- Trouble breathing
- Anything that worries you

Card 4, the 2 month visit

Approximate age: 8 weeks.

A note: This is usually the first visit with multiple vaccines. The pediatrician will explain the schedule for your visit. This planner does not name specific vaccines.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ The full physical exam
- ☐ Social smile (often emerging now)
- ☐ Following objects with the eyes
- ☐ Cooing
- ☐ Pushing up briefly during tummy time
- ☐ Feeding and sleep
- ☐ Vaccines (per the current schedule, your pediatrician will tell you)

WHAT TO PREPARE:

- ☐ A list of what they are doing developmentally that you want to mention:

- ☐ What feeding looks like right now: _____
- ☐ Sleep pattern (rough): _____
- ☐ Questions about vaccines I want to ask: _____
- ☐ My top question: _____

WHAT TO ASK:

- Which vaccines are scheduled today, and what are they protecting against?
- What side effects can I expect tonight and tomorrow?
- What should I give for a fever, and at what threshold? (Confirm with your provider, not the internet.)
- Are there any developmental things I should be doing more of?
- When is the next visit?

WHAT THEY SAID:

Weight: _____ Percentile: _____

Length: _____ Percentile: _____

Head: _____ Percentile: _____

Vaccines given today (their list):

What they said about expected side effects:

What they recommended for fever:

Development notes:

Next visit: _____

RED FLAGS (especially after vaccines):

- A fever lasting longer than the timeframe they told you, or higher than they told you
- Inconsolable crying for several hours
- Difficulty breathing
- Lethargy or unresponsiveness
- A new rash other than at the injection site
- Any reaction the pediatrician told you to call about

Card 5, the 4 month visit

Approximate age: 16 weeks.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ Holding head steady, pushing up well
- ☐ Reaching for objects
- ☐ Laughing, babbling
- ☐ Recognizing faces
- ☐ Rolling, maybe (front to back is often first)
- ☐ Sleep and feeding
- ☐ Vaccines per schedule
- ☐ Conversation about when to think about solids (timing varies by provider)

WHAT TO PREPARE:

- ☐ Developmental milestones they have hit: _____
- ☐ Any developmental concerns: _____
- ☐ Sleep pattern: _____
- ☐ Feeding pattern: _____
- ☐ My top question: _____

WHAT TO ASK:

- How are they developing?
- When do you typically recommend starting solids?
- (If exclusively breastfeeding) Any supplements you recommend at this age?
- What vaccines are today?
- Anything I should be watching for before the 6-month visit?

WHAT THEY SAID:

Weight: _____ Length: _____ Head: _____

Vaccines today:

What they said about starting solids:

What they said about supplements:

Other notes:

Next visit: _____

RED FLAGS:

- Not making eye contact
- Not responding to sound
- Not pushing up at all during tummy time
- Crossed eyes consistently after this age
- Persistent fever, vomiting, refusing feeds

Card 6, the 6 month visit

Approximate age: 24 weeks.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ Sitting with support
- ☐ Rolling both ways
- ☐ Reaching, transferring objects between hands
- ☐ Babbling with consonants
- ☐ Stranger awareness sometimes emerging
- ☐ Teeth or signs of teeth
- ☐ Solid food introduction
- ☐ Vaccines per schedule

WHAT TO PREPARE:

- ☐ How solids (if started) are going: _____
- ☐ Any foods that seemed to cause a reaction: _____
- ☐ Sleep changes lately: _____
- ☐ Developmental milestones: _____
- ☐ My top question: _____

WHAT TO ASK:

- Is their growth on track?
- How should I be approaching solids over the next 3 months?
- Any concerns about allergens? When and how to introduce?
- Vaccines today, and side effects to expect?
- Sleep at this age, what is realistic to expect?

WHAT THEY SAID:

Weight: _____ Length: _____ Head: _____

Solids guidance:

Allergens guidance:

Vaccines today:

Development notes:

Next visit: _____

RED FLAGS:

- Not sitting with any support
- Not vocalizing
- Not interested in faces
- Persistent fever, refusing feeds, lethargy
- Allergic reaction to a food, hives, swelling, breathing changes (call urgently)

Card 7, the 9 month visit

Approximate age: 36 weeks.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference
- ☐ Sitting independently
- ☐ Crawling (or scooting, or rolling places, all are normal variations)
- ☐ Pulling to stand
- ☐ Babbling with variety
- ☐ Object permanence (peekaboo lands)
- ☐ Pincer grasp emerging
- ☐ Stranger / separation awareness
- ☐ Solid food progress
- ☐ Iron / hemoglobin sometimes checked at this visit

WHAT TO PREPARE:

- ☐ How sleep is going: _____
- ☐ How eating is going: _____
- ☐ Any milestones they hit recently: _____
- ☐ Any milestones you are watching for: _____
- ☐ My top question: _____

WHAT TO ASK:

- Are they on track for crawling and pulling up?
- Any iron / hemoglobin recommendations?
- What should they be eating at this stage?
- What's typical for sleep at 9 months?
- When is the next visit?

WHAT THEY SAID:

Weight: _____ Length: _____ Head: _____

Bloodwork (if any):

Feeding guidance:

Sleep guidance:

Other notes:

Next visit: _____

RED FLAGS:

- Not sitting independently
- Not babbling
- Not responding to their name
- Loss of skills they previously had
- Persistent illness symptoms

Card 8, the 12 month visit (1 year)

Approximate age: 12 months.

WHAT THEY USUALLY CHECK:

- ☐ Weight, length, head circumference (length transitions to height around now)
- ☐ Pulling up, cruising, maybe standing alone
- ☐ A word or two, maybe more
- ☐ Pointing
- ☐ Waving
- ☐ Drinking from a cup
- ☐ Iron / hemoglobin / lead screening at some practices
- ☐ Vaccines per schedule
- ☐ Transition from formula or breast milk to cow's milk (if applicable, per their guidance)

WHAT TO PREPARE:

- ☐ Any words they say: _____
- ☐ Sleep pattern: _____
- ☐ Eating pattern: _____
- ☐ Milestones in motor skills: _____
- ☐ My top question: _____

WHAT TO ASK:

- Any switch from breast milk / formula to cow's milk, and when?
- Iron and lead screening, do we need either today?
- Vaccines today?
- Sleep at 1 year, what is typical?
- When is the next visit (15 months in many practices)?

WHAT THEY SAID:

Weight: _____ Length/Height: _____ Head: _____

Bloodwork (if any):

Milk transition guidance:

Vaccines today:

Other notes:

Next visit: _____

RED FLAGS:

- No words by 12 to 15 months (worth mentioning)
- Not pulling up
- Loss of previously gained skills
- Persistent illness or behavior change

Card 9, the 15 month visit

Approximate age: 15 months.

WHAT THEY USUALLY CHECK:

- ☐ Weight and height
- ☐ Walking, with some confidence
- ☐ A few words
- ☐ Following one-step directions
- ☐ Pointing to body parts (some)
- ☐ Vaccines per schedule

WHAT TO PREPARE:

- ☐ Their words: _____
- ☐ Their walking: _____
- ☐ Their eating: _____
- ☐ My top question: _____

WHAT TO ASK:

- Are they on track for language?
- Is their walking pattern okay?
- Any tantrums or behavior I should know are normal at this age?
- Vaccines today?
- Next visit?

WHAT THEY SAID:

Weight: _____ Height: _____

Language notes:

Motor notes:

Vaccines:

Other:

Next visit: _____

RED FLAGS:

- Not walking by 18 months (worth mentioning)
- No words
- Significant behavior or sleep change with no explanation

Card 10, the 18 month visit

Approximate age: 18 months.

A note: Many practices do a developmental screening at this visit.

WHAT THEY USUALLY CHECK:

- ☐ Weight, height
- ☐ Walking confidently, climbing
- ☐ Vocabulary: many words, beginning to combine two
- ☐ Pointing to objects
- ☐ Following simple instructions
- ☐ Pretend play emerging
- ☐ Developmental screening (often the M-CHAT or similar at this visit)
- ☐ Vaccines per schedule

WHAT TO PREPARE:

- ☐ Words they say (rough count): _____
- ☐ Whether they combine two words: _____
- ☐ Tantrums: pattern and severity: _____
- ☐ My top question: _____

WHAT TO ASK:

- How did the developmental screening look?
- Any concerns I should think about?
- Is the language on track?
- Vaccines today?
- Next visit?

WHAT THEY SAID:

Weight: _____ Height: _____

Developmental screen result, in their words:

Language:

Vaccines:

Other:

Next visit: _____

RED FLAGS:

- No words
- Loss of previously used words
- No response to their name
- Loss of social skills they had
- Any concern that surfaced at the developmental screen, write it down

Card 11, the 24 month visit (2 year)

Approximate age: 24 months.

WHAT THEY USUALLY CHECK:

- ☐ Weight, height, BMI calculation now begins
- ☐ Vocabulary: many words, combining 2 to 4
- ☐ Following multi-step instructions
- ☐ Running, climbing, kicking
- ☐ Pretend play
- ☐ Beginning to assert independence (very age-appropriate)
- ☐ Developmental screening (M-CHAT or similar)
- ☐ Vaccines per schedule
- ☐ Conversation about toilet learning readiness (no rush)

WHAT TO PREPARE:

- ☐ Vocabulary update: _____
- ☐ Toilet learning interest, if any: _____
- ☐ Sleep pattern: _____
- ☐ Eating pattern: _____
- ☐ My top question: _____

WHAT TO ASK:

- How is development tracking?
- Anything about screen time recommendations at this age?
- Toilet learning, when is right for us?
- Vaccines today?
- Next visit (often at 2.5 years, or annual after this)?

WHAT THEY SAID:

Weight: _____ Height: _____ BMI: _____

Developmental notes:

Toilet learning guidance:

Vaccines:

Other:

Next visit: _____

RED FLAGS:

- Significantly fewer words than typical (your pediatrician will be specific)
- Not combining words
- Loss of skills
- Significant behavioral changes

Card 12, the 30 month visit (2.5 year, in many practices)

Approximate age: 30 months.

A note: Not every practice does a separate 30-month visit. If yours does not, this card is empty and you can skip it.

WHAT THEY USUALLY CHECK:

- ☐ Weight, height
- ☐ Language: hundreds of words, multi-word sentences
- ☐ Motor: jumping, climbing stairs alternating feet (sometimes)
- ☐ Pretend play, social interaction
- ☐ Developmental screening
- ☐ Vaccines per schedule (often few or none at this visit)

WHAT TO PREPARE:

- ☐ What you have noticed about their language: _____
- ☐ Their social play with other kids: _____
- ☐ Toilet learning status: _____
- ☐ My top question: _____

WHAT TO ASK:

- Development tracking?
- Anything to watch for before age 3?
- Toilet learning, where we are?
- Anything social or behavioral worth discussing?

WHAT THEY SAID:

Weight: _____ Height: _____ BMI: _____

Notes:

Next visit: _____

When to call between visits

The cards cover scheduled visits. This page is for the calls between.

CALL THE PEDIATRICIAN'S OFFICE IF:

For a baby under 3 months:

- ☐ Any fever (any elevated temperature warrants a call)
- ☐ Trouble breathing, fast or noisy breathing
- ☐ Refusing to feed
- ☐ Significantly fewer wet diapers than usual
- ☐ Lethargic, hard to wake
- ☐ A rash that is spreading, especially if it does not blanch under pressure
- ☐ Inconsolable crying for more than an extended stretch

For an older baby or toddler:

- ☐ A fever above the threshold your pediatrician set, lasting longer than they told you
- ☐ Vomiting that does not stop, especially with fewer wet diapers
- ☐ Persistent diarrhea, especially with fewer wet diapers
- ☐ Trouble breathing, fast or noisy breathing, wheezing
- ☐ A rash that is spreading or does not blanch under pressure
- ☐ Lethargic, very floppy, hard to wake
- ☐ A head injury where you are not sure
- ☐ Possible exposure to medication, cleaning product, or unknown substance
(Poison Control is also a fast call: in the US, 1-800-222-1222)
- ☐ Any allergic reaction (hives, swelling, breathing changes)
- ☐ Loss of skills they had before
- ☐ A change in behavior that worries you

For yourself:

- ☐ Postpartum mood is heavy, lasting longer than the first few weeks
- ☐ Thoughts of harm (yours or theirs)
- ☐ A sense you cannot do this

These warrant a call to your own provider or the perinatal mental-health line, not the pediatrician, but the pediatrician is also a resource.

NUMBERS I WILL CALL:

Pediatrician's office: _____

After-hours line: _____

Poison Control (US: 1-800-222-1222): 1-800-222-1222

Nearest pediatric ER: _____

My own provider's office: _____

A final note

Twelve cards. Two years. By the end of it, you will have a record of every measurement, every recommendation, every conversation that mattered.

It is also a record of you. The questions you brought to each visit. The things you noticed at home and remembered to mention. The dates you wrote down because you knew you would not remember them otherwise.

Keep the filled cards in a folder. The pediatrician changes practices, you move, you switch insurance. The folder is yours.

You are doing the work. They are growing.

With care, Maya



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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